

Question 1

NOT COMPLETE

The evening nurse in the neurosurgical unit is caring for a 55-year-old postoperative client.

Health HistoryNurses' NotesLaboratory ResultsDiagnostic Tests

1000:

Client returns from the OR. Sleepy but easily arousable and oriented. Answering questions appropriately. Able to move all extremities. Intact sensory perception. Head of bed elevated at 30°, head midline. No drainage on the head dressing. Glasgow Coma Scale (GCS) = 15, PERRLA.
Oxygen at 2 L via NC, SpO₂ 96%. T 37.2° C (99.0° F), HR 82 BPM and regular, RR 20 bpm, BP 130/80 mm Hg, NSR on cardiac monitor.

1500:

Evening nurse performs an assessment 5 hours postoperatively. Client is difficult to arouse, opening eyes only to firm stimulation; oriented to name only. Speech is slurred and hesitant, with difficulty finding words when answering questions. New weakness noted in the left upper and lower extremities, with a weaker hand grip on the left. Decreased response to light touch on the left side. Pupils are sluggish to react to light bilaterally. Client reports nausea and a headache rated 7/10 and has vomited once, non-projectile. Periorbital edema and ecchymosis noted bilaterally around the eyes. Head dressing intact; a small (dime-sized) amount of dark red, dried drainage noted on the dressing, marked and dated for baseline comparison per agency practice. Head of bed elevated at 30°, head midline.
Oxygen at 2 L via NC, SpO₂ 95%. T 38.1° C (100.6° F), HR 58 BPM and regular, RR 10 bpm and irregular, BP 168/62 mm Hg, NSR on cardiac monitor. Glasgow Coma Scale (GCS) = 12.

The evening nurse in the neurosurgical unit performs an assessment on a 55-year-old postoperative client who returned to the unit 5 hours ago following open craniotomy via supratentorial surgery to remove a brain tumor. The nurse reviews the immediate postoperative nurses' notes from 1000 and the neurological flow sheet documentation done every 30 minutes by the day nurse prior to performing an assessment and notes no significant changes.

- Complete the diagram by dragging from the choices below to specify what condition the client is most likely experiencing, 2 actions the nurse should take to address that condition, and 2 parameters the nurse should monitor to assess the client's progress.

Action to Take

Action to Take

Condition Most Likely Experiencing

Parameter to Monitor

Parameter to Monitor

Actions to Take	Potential Condition	Parameters to Monitor
Notify the surgeon	Stroke	White blood cell count
Obtain blood cultures	Bleeding	Intake and output
Request a prescription for IV antibiotics	Infection	Swallowing ability
Request a prescription for packed red blood cells (PRBCs)	Increased intracranial pressure	Hemoglobin and hematocrit levels
Request a prescription for an IV hyperosmotic agent		Vital signs and neurological status

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Health History	Nurses' Notes	Laboratory Results	Diagnostic Tests
Category	Findings		
Reason for Admission	2 months of progressively worsening headaches and blurred vision; MRI revealed a right frontal brain mass. Admitted for elective open craniotomy via supratentorial approach for tumor resection.		
Past Medical History	Hypertension, well-controlled with lisinopril. No known bleeding disorder. No known drug allergies.		

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CORRECT

Score: 5 / 5 points